

Study Summary

Study Summary The skin and systemic disease Clues to possible systemic disease: I

Rash ass. with joint pain, fever, weight loss, weakness, SOB, altered bowel function I

Rash not responding to topical tx I

Erythema of the skin (inflammation around blood vessels)

I Non blanching palpable purplish fixed lesion , painful and blistering >> vasculitis I

Unusual changes in pigmentation or texture of the skin.

I Palpable dermal lesion 2ry to granuloma , mets , lymphoma and so on...

Characteristic rash that indicates underlying systemic disease : I

Erythema multiforme >> HSV, Hep B/C, mycoplasma pneumonia I

Pyoderma gangrosum >> IBD , RA , hematological malignancy I

Erythema nodosum >> IBD , streptococcal infxn , TB , sarcoidosis , behcet's I

Vasculitis >> Hep B /C , SLE , lymphoma , leukemia Skin reactions ass with infections

1. Toxic erythema I

Widespread symmetrical Maculopapular erythema (morbilliform)

I starts on the trunk then spreads to the limbs

I Usually asymptomatic (itchy)

I Blanching I

Triggered by viruses, bacteria, drugs I

Classic presentation??

I Tx: No need, can use; Iemollient and topical steroid if symptomatic Itreat the underlying disease

Toxic Erythema

2. EM I

Epidemiology I

Presentation : target lesions I

Pathophysiology: Type IV HSR I

Classification : EM Minor vs Major I

Causes: according to classification, most common cause: HSV I

Dx : Clinical I

TX I

Recurrence ?

I Coarse

I Rare associations EM I

The most common infectious trigger HSV I

Systemic symptoms of the underlying infection usually precede the EM rash by 2–14 days.

EM Doll's eye Acrallyftandstar

3 shades Dusky red residual necrosis sometimes feet of colors pinkish Red again HSV

5 14 days Erythema Nodosum I

Most common Panniculitis.

I Epidemiology: F >> M I

Presentation: acute, painful nodules (Shin), especially by palpation.

I Idiopathic (50%) vs Secondary I

Secondary causes: Multiple, shown in the table I

Most Common Sec : Strep I

Skin lesions: very tender nodules (3–20 cm), not sharply marginated I

Dx & management: Bacterial Culture, Imaging, Dermatopathology I

Course: Spontaneous resolution occurs in 6 weeks I

Tx: treat secondary; rest + NSAIDs (steroids ?)

Panniculitis adipose tissue inflammation

3. Erythema nodosum T der Susa b nodules Streptococci O over the shin Yersinia multiple
bilateral stenosomiasis A 20 301 idiopathic need to exclude any causes most commonly ifx n EN:
Causes

4. Erythema annulare centrifugum Vs Tinea Corporis comitugically amenity Ya Tinea pedis
idiopathic gppffichongi

5. Erythema chronicum migrans I

Caused by Borrelia burgdorferi (Lyme disease)

I Migrating erythema I

Cutaneous inflammatory response to Borrelia.

h spirochete like syphilis The transmitted Yarger C sign began hematogenous spread Ws Cns Au
like symptoms Sarcoidosis I

Unknown etiology, atypical mycobacterium may be the trigger I

Presentation: I

May occur with or without cutaneous disease I

Skin lesions : Specific vs Non-specific I

Earliest: skin-colored papules, on the face, heal without scarring.

I Then: Plaques; annular, extremities and Buttocks, SCARRING, Chronic disease I

Lupus Pernio: violaceous plaques on the nose and cheeks I

EN: m/c NS skin lesion, early sarcoid.

I

Scarring sarcoidosis.

Sarcoidosis CXR Bilateral hilar lymphadenopathy Noncaseating Tubercle TB Multisystemic

20 cutaneous Bigmimic Sarcoidosis Perianths scalytagy Dusky infiltrated lesions: Lupus Pernio
Badprognosticsign URI IRI osteolytic AcrallyThose bison dusky violaceous color Lupus pernio vs
Rosacea midfacial erythema Sharpdermaltatientviolaceous hue Scarring sarcoid DDx Tififial
hypertrophicpre existingscar sarcoid Löfgren syndrome

-Erythema nodosum

-Fever

-Arthritis

-Hilar adenopathy Acutesyndrome multisystemic Benign resolvesspontaneously

É

Diagnosis and management I

Imaging I

Labs I

Lesional biopsy I

Tx: I

Systemic involvement: systemic steroids I

Limited cutaneous: topical steroids, if extensive: systemic steroids, mtx, hydroxychloroquine.

Skin changes ass with hormonal imbalance Hyperpigmentation I

Increase in circulating hormones with melanocytestimulating activity

I Hyperthroidism, acromegaly, Addison's disease I

Pregnancy, OCPs -> melasma/chloasma (localized on the forehead and cheeks)

I It may fade slowly if ultraviolet light is excluded from the affected skin using daily sun block.

Generalized Addison mulousmen localized creases melasmalyspiotsumprotection E

Hypopigmentation I

Widespread partial loss of melanocyte function I

Hypopituitarism (absence of MSH)

Acanthosis nigricans noBlack Insulin resistance axilla inguinal neck G Increasedthickness of epidermis obese areas extensive involvement Gastric adenocarcinoma Necrobiosis lipidica I Associated with DM I

Not related to the severity of DM I

Coarse not affected by controlling blood suger I

Tx: intralesional steroids, surgery.

destructionofcollagen indermis Atrophy translucent Brs yellowishhue depositing

654Épplwith CheckHgbac Thyroid disease edema pitting Ohridshift I Ettionpitting Grave's discs Generalizedmyxedema Graves disease I

Hyperthyroidism with diffuse goiter, Ophthalmopathy and dermopathy.

I

Dermopathy: pretibial myxedema I

Tx: steroids Skin changes with GI and liver diseases I

Acrodermatitis enteropathica : genetic disorder of Zinc absorption (seen in neonates)

I Acquired zinc deficiency (AZD) occurs in older individuals due to dietary deficiency or failureof intestinal absorption of zinc (malabsorption,alcoholism, prolonged parenteral nutrition)

I Skin changes usually appear within weeks of birth with erythematous inflamed scaly skin around the mouth, anus and eyes Zinc deficiency Breastfed full ATTY ofzin inthetransport ofzinc likeeczema orifices q atrally genetic to therapy minedmalabsorption notresponding Zinc def I

If the condition is not recognised and treated promptly then the skin can become crusted, eroded and secondarily infected.(candida, S.aureus)

I Other findings: I

Zinc supplementation (1mg/kg/day), should be continued until zinc levels normalise (or lifelong in inherited forms).

Vitamin C deficiency I occurs in those with malabsorption problems, those on a poor diet, the elderly and alcoholics I

Vascular fragility I

Skin lesions: Petechiae, follicular hyperkeratosis with perifollicular hemorrhage, especially on the lower legs I

Other findings: I

Labs: Normocytic, normochromic anemia, Serum ascorbic acid level zero. X-ray findings are diagnostic I

Tx: Ascorbic acid imphcollagensynthesis Satb A Hyperplastic easybleeding afterteethbrushing abnormalhair corkscrew bleeding in hair follicle

3. Pyoderma gangrenosum I

Rapid and painful ulcerated necrotic skin areas with hypertrophic undermined purplish margins.

Assoc with UC, Crohn's, RA, leukemia, monoclonal gammopathy.

look for any cause Tibial trauma TT papule pustule

4 rapid and sudden. OPG

4. Dermatitis herpetiformis Buttons disease Celiac disease extensor surfaces highly pruritic

5. Liver disease Porphyria cutanea tarda according to which enzyme of heme metabolism is affected there is synthesis defect and all porphyrias have cutaneous manifestations porphyrins accumulate Asensitivity to sunlight recurrent inflammation cause scarring milium Xanthomas Me full of lipids due to genetic disease a yellowish lipid O Hypopigmentation disorders I

Albinism, AR, loss of pigment of skin, eyes and hair, no pigment production I

Pioidism, AD, triangular hypopigmented patches, disorder of melanocyte development I

Vitiligo localized depigmentation, sharply demarcated, symmetrical macular lesions, loss of melanocytes and melanin I

Post-inflammatory such as psoriasis, eczema, lichen planus and lupus erythematosus; infections, chemicals, reactions to pigmented naevi, genetic diseases D/D Congenital there are melanocytes defect've Tsinase enzyme eye problems nystagmus forehead forehead

Desmith' enof melanocytes Acquired

Hyperpigmentation disorders There is wide variation in the pattern of normal pigmentation as a result of hereditary factors and exposure to the sun.

Darkening of the skin may be due to:

1. An increase in the normal pigment melanin I

2. Deposition of bile salts from liver disease I

3. Iron salts (haemochromatosis)

I

4. Drugs or metallic salts from ingestion I

5. AN is characterised by darkening and thickening of the skin of the axillae, neck, nipples and umbilicus I

6. Post-inflammatory pigmentation is common, often after acute eczema, fixed drug eruptions and lichen planus.

I

7. In malabsorption syndromes such as pellagra and scurvy, there is commonly increased skin pigmentation.

Skin changes of underlying malignancy Mycosis fungoides In cutaneous T-cell lymphoma different clinical presentation Poikiloderma (telangiectasia, reticulate pigmentation)

I Hyper Hypopigment telangiectasia radiotherapy T-cell idiopathic atrophy Skin and Pregnancy I

Prurigo gestationis- is a benign non-specific pruritic (itchy) papular rash that arises during pregnancy and is generally more severe in the first trimester.

I Polymorphous eruption of pregnancy (PEP) is a pruritic erythematous rash that usually starts in the striae of the abdomen during the third trimester and can become widespread. No effect on baby I

Pemphigoid gestationis (PG) is a rare disorder that may initially resemble PEP but develops pemphigoid-like vesicles, spreading over the abdomen and thighs (Figure 10.23). PG is an autoimmune disorder, (Small baby and higher mortality).

Egg low weight low gestahage PEP poly not vesicles eczematous papules duetotension Fina umbilical region no recurrence Pemphigoid gestations

0 Buttons pemphigoid o periumbilical Baby can be affected t recurrence GOOD LUCK